

AI-powered modernization compresses time and risk

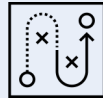


Value first

- We rank every system by business value, not technical elegance.
- Cost, risk and revenue exposure scored side by side.
- Quick wins funded first, so savings pay for later waves.
- Value tracked every release.

WHY THIS MATTERS

Effort goes where return is highest, so promised value lands.



Purpose built

- Retain, replatform, rewrite or retire, chosen per system.
- Stable components stay in place, untouched.
- Target architecture fits your portfolio and skills.
- Nothing is rebuilt for show.

WHY THIS MATTERS

You keep what works and replace only what drives value.



AI accelerated

- Agents scan and map codebases at scale.
- Dependencies and dead code surface in days.
- Generated tests and docs back each change.
- Engineers approve every merge.

WHY THIS MATTERS

Months of analysis shrink to days, with less tribal knowledge.

AI-powered modernization compresses time and risk

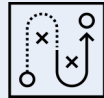


Value first

- Rank work by business value.
- Weigh cost, risk and revenue together.
- Fund quick wins first.
- Track value per release.
- Review progress with sponsors each quarter.

WHY THIS MATTERS

Effort goes where return is highest.



Purpose built

- Choose retain, replatform, rewrite or retire.
- Keep stable components in place.
- Fit the target to your portfolio.
- Nothing rebuilt for show.
- Plan system by system.

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WHY THIS MATTERS

Months of analysis done in days.



Co-build and scale

- Co-sourced teams sit with stakeholders.
- Onshore and nearshore flex with demand.
- Pairing builds your team's skills.
- Handover is planned from day one.

WHY THIS MATTERS

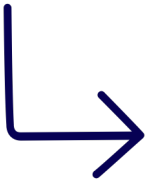
Capacity without disruption.

AI-powered modernization compresses time and risk



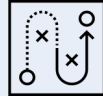
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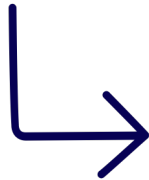
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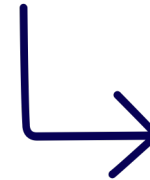
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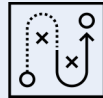


Value first

- **Ranked by value.** Cost, risk and revenue exposure scored per system.
- **Quick wins first.** Fund later waves.
- **Dependencies weighed.** Complexity enters every sequencing call.
- **Per release.** Value measured.
- **Business owned.** Quarterly sign-off.

WHY THIS MATTERS

Effort goes where return is highest, so promised value lands.



Purpose built

- **Four paths.** Retain, replatform, rewrite or retire, per system.
- **Stable stays put.** Working parts stay.
- **Fit to portfolio.** Target matches your skills and vendors.
- **Evidence based.** Usage, defects, cost.
- **No rebuild for show.** Return first.

WHY THIS MATTERS

You keep what works and replace only what drives value.



AI accelerated

- **Scan at scale.** Agents map code, data flows and dependencies.
- **Days, not months.** Discovery in weeks.
- **Tests generated.** Every change ships with tests and docs.
- **Humans decide.** Engineers review each merge.
- **Knowledge kept.** Findings in a shared record.

WHY THIS MATTERS

Months of analysis shrink to days, with less tribal knowledge.

OUR APPROACH

AI-powered modernization compresses time and risk

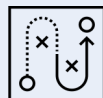


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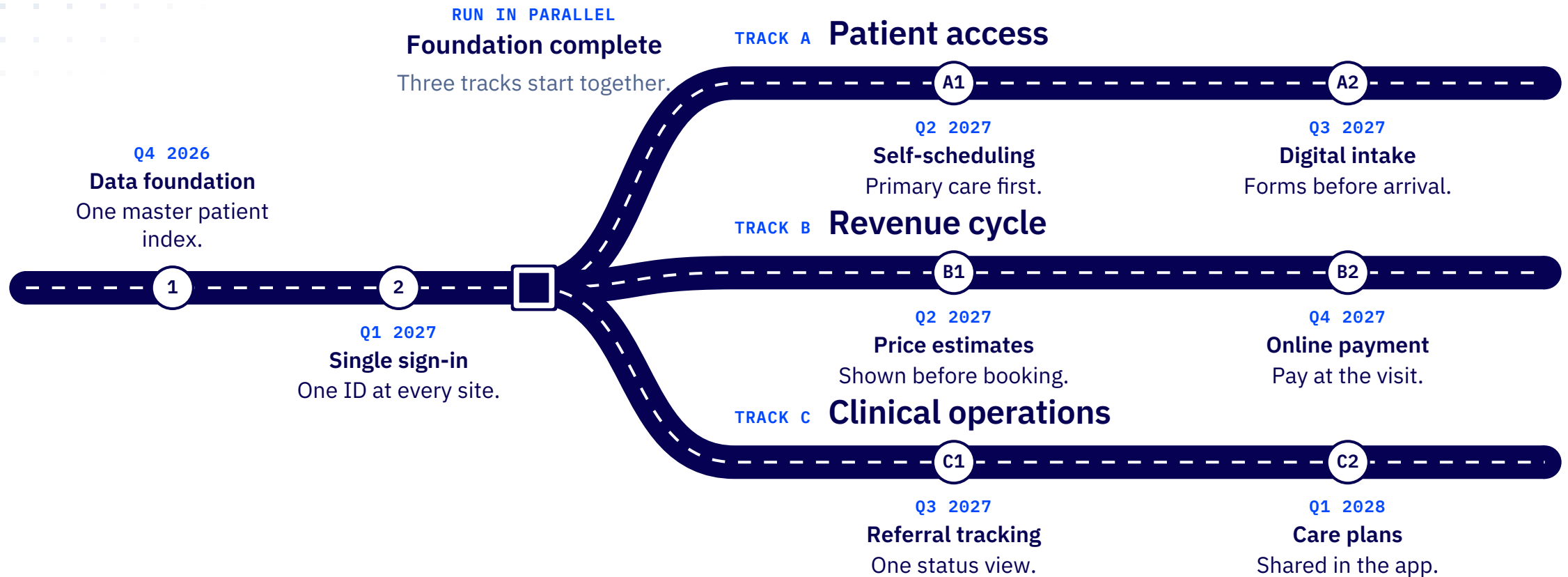
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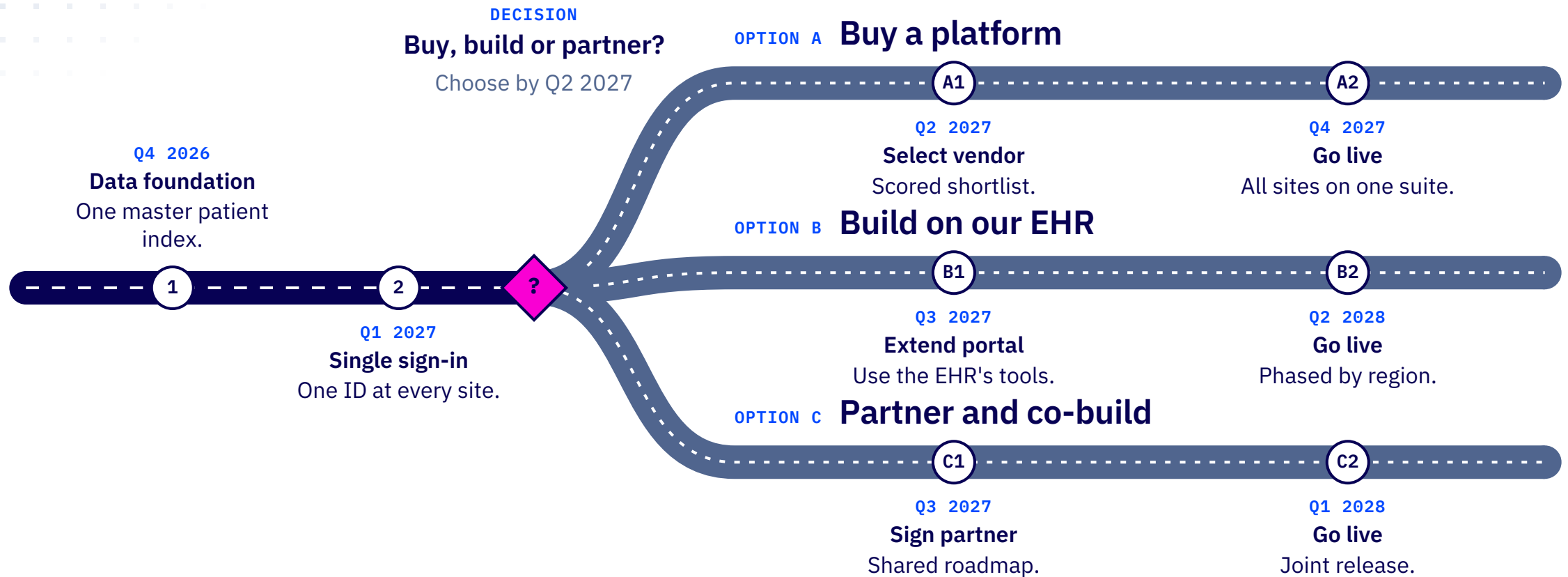
Months of analysis shrink to days, with less tribal knowledge.

One foundation, then three tracks run in parallel



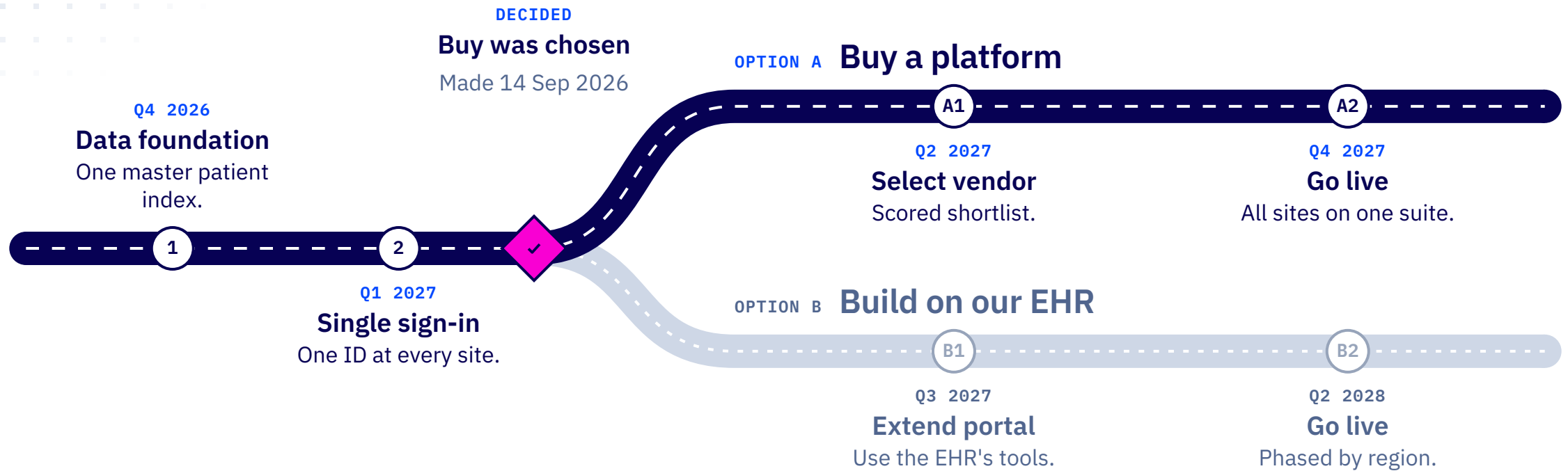
Source: Northfield Health System digital front door program.

You must choose: buy, build or partner



Source: Northfield Health System digital front door program.

We chose to buy a platform



WHY WE CHOSE TO BUY

Fastest to every site, lowest integration risk, and a vendor roadmap we do not have to fund.

Source: Northfield Health System digital front door program.

ROADMAP

Three tracks share one foundation

The foundation must land first. After that, access, revenue and clinical teams ship independently.

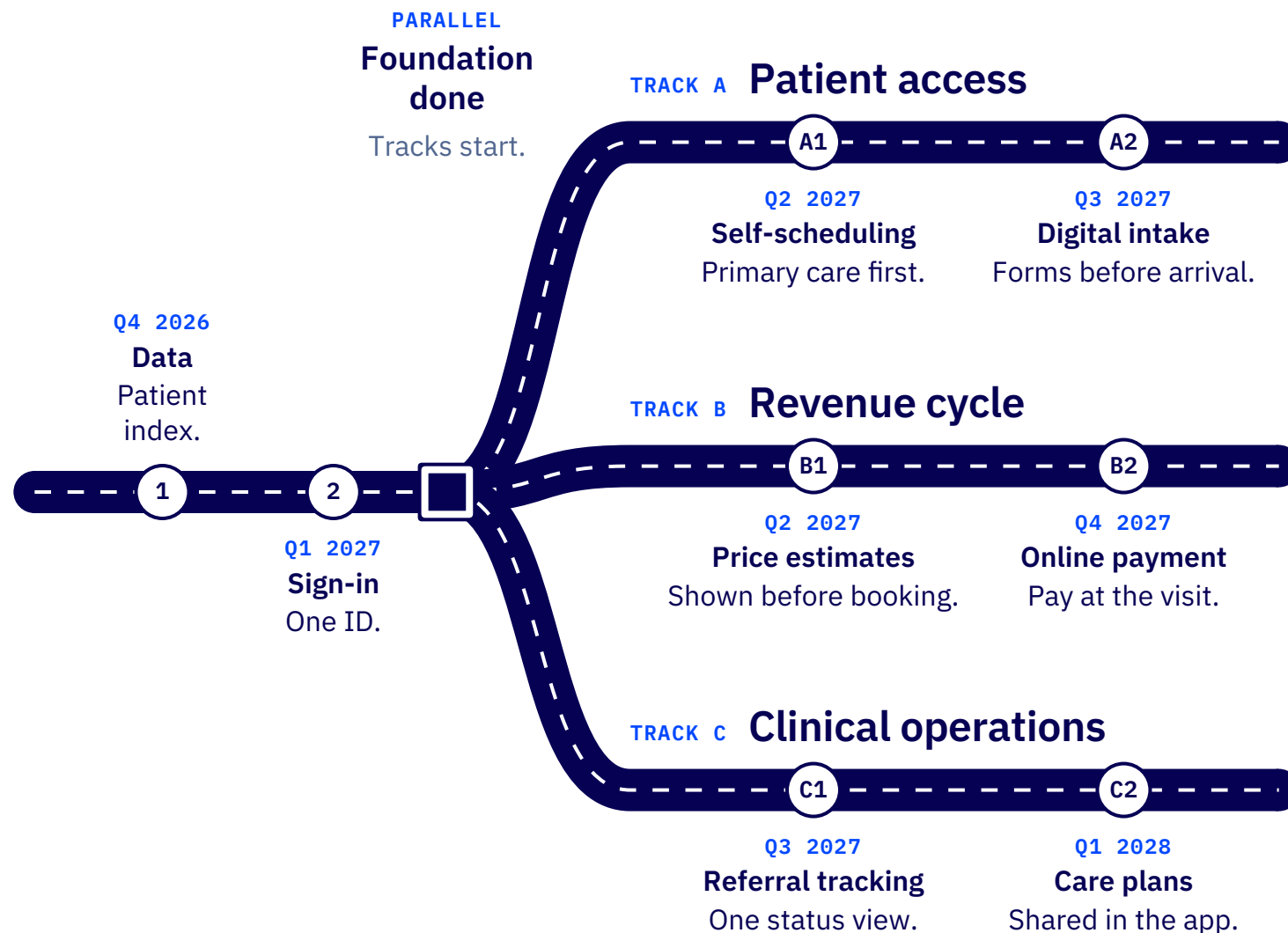
TRACK OWNERS

- **Access.** Chief patient access officer.
- **Revenue.** VP revenue cycle.
- **Clinical.** Chief nursing informatics officer.

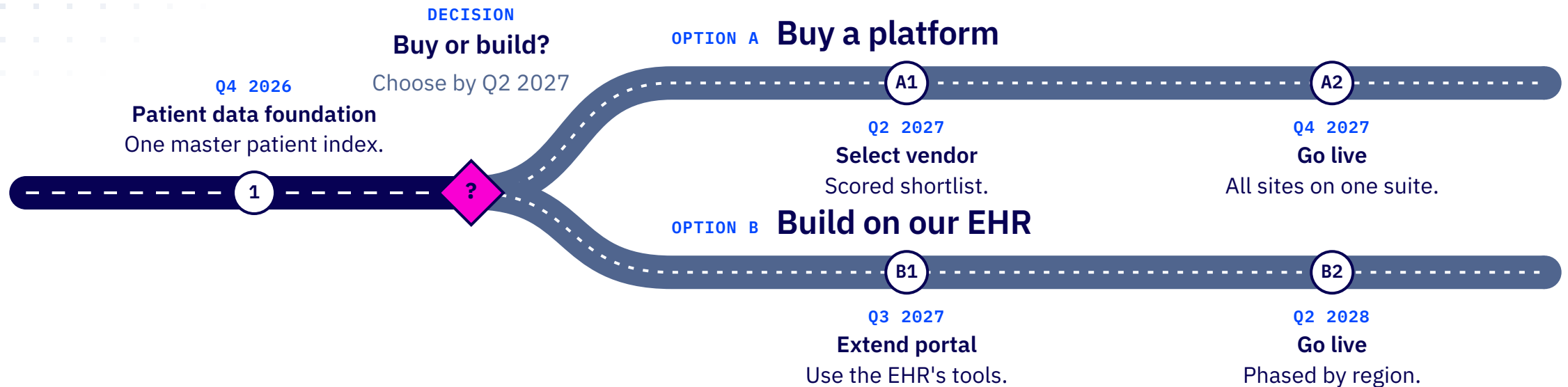
BIGGEST DEPENDENCY

Single sign-in gates all three tracks.

Source: Northfield Health System digital front door program.



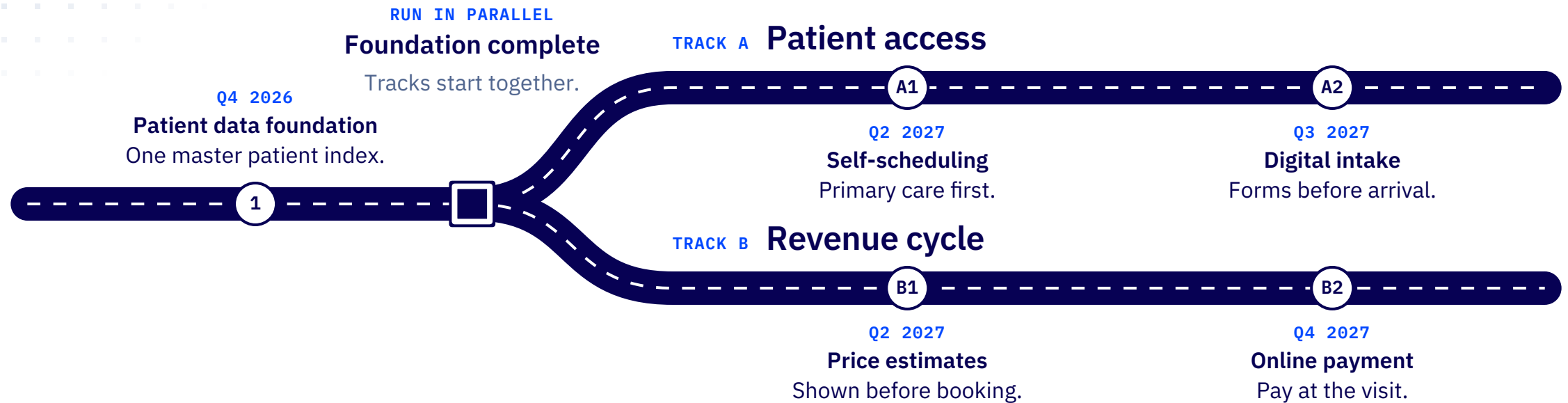
The choice turns on time and fit, not cost



CRITERION	A BUY A PLATFORM	B BUILD ON OUR EHR
Cost to Northfield	Medium	Strong
Time to every site	Strong	Weak
Delivery risk	Strong	Medium
Fit with clinical workflow	Medium	Strong

Source: Northfield Health System digital front door program.

Each track has its own outcome, owner and dependency



TRACK A • PATIENT ACCESS

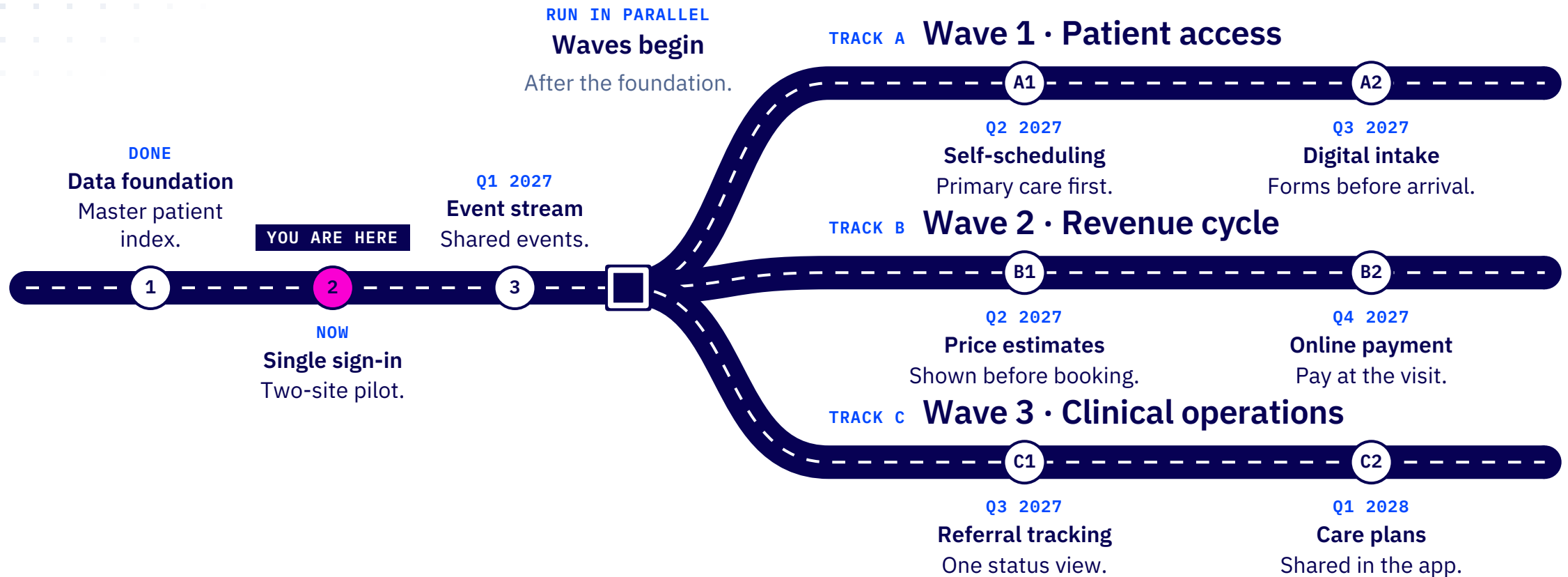
- **Outcomes.** 70% of primary care visits booked online.
- **Owner.** Chief patient access officer, with the access squad.
- **Depends on.** Master patient index; single sign-in.

TRACK B • REVENUE CYCLE

- **Outcomes.** Estimates on 90% of scheduled visits.
- **Owner.** VP revenue cycle, with the billing squad.
- **Depends on.** Payer contract data loaded.

Source: Northfield Health System digital front door program.

The foundation is half built; the waves follow



Source: Northfield Health System digital front door program.

Three horizons build one front door

HORIZON	OBJECTIVE	KEY INITIATIVES	OUTCOMES AND KPIS	DEPENDENCIES	INVESTMENT
Now Q4 2026 to Q1 2027	Lay the data foundation so every system agrees who the patient is.	- Master patient index - Single sign-in, patients and staff - Scheduling event stream	- Duplicates down 40% - One ID at 100% of sites	- EHR release 4.2 - Privacy sign-off	\$4.2M
Next Q2 to Q3 2027	Open the front door: patients book, register and see prices online.	- Self-scheduling in primary care - Digital intake and consent - Price estimates before booking	60% of visits booked online - Call volume down 25%	- Patient index live - Payer contract data loaded	\$6.8M
Later Q4 2027 to 2028	Extend access across the care journey and personalize it.	- Referral tracking end to end - Care plans in the app - Virtual front desk and chat	- No-show rate under 6% - App use by 50% of patients	- Event stream - Clinical capacity	\$5.5M

Source: Sample figures for layout only; replace with current firm data before use.

Why the roadmap runs in this order

Patients feel the front door, but it only works once the data underneath it is trusted.

- **Identity first.** Every later feature needs one patient ID.
- **Access before payment.** Booking creates the demand that estimates and billing serve.
- **Care last.** Clinical workflows change slowest, so we start them early but ship them last.

Source: Northfield Health System digital front door program.

NOW • Q4 2026 TO Q1 2027

WHAT

One master patient index and one sign-in across all six hospitals.

WHY NOW

Two registration flows create duplicates and denials today.

UNLOCKS

Self-service for every digital feature that follows.

NEXT • Q2 TO Q3 2027

WHAT

Online scheduling, digital intake and price estimates.

WHY NOW

Patients ask for it, and competitors already offer it.

UNLOCKS

Fewer calls, fewer no-shows and cleaner claims.

LATER • Q4 2027 TO 2028

WHAT

Referral tracking, care plans and a virtual front desk.

WHY NOW

Not yet: it needs the event stream and clinical capacity.

UNLOCKS

Care that continues between visits.

Each wave starts and ends on evidence, not on the calendar

WAVE	SCOPE	ENTRY CRITERIA	EXIT CRITERIA	DECISION GATE
Wave 1 Foundation	- Master patient index - Single sign-in	- Funding approved - Privacy review signed	- Duplicates down 40% - Sign-in live at all sites	Access council confirms the data is trusted
Wave 2 Access	- Self-scheduling - Digital intake	- Wave 1 exit met - Two pilot clinics named	60% of visits booked online - Intake done for 50%	Executive team chooses buy or build
Wave 3 Revenue	- Price estimates - Online payment	- Payer data loaded - Billing squad staffed	- Estimates on 90% of visits - Denials down 15%	CFO signs off on rollout to all sites
Wave 4 Clinical	- Referral tracking - Care plans	- Event stream at scale - Clinical lead named	- Every referral tracked - Care plans in 3 service lines	Clinical council approves workflow changes

Source: Northfield Health System digital front door program.

Three objectives, six initiatives, one owner each

	INITIATIVE	WHAT IT DELIVERS	OWNER	START	LIVE	HORIZON
KNOW PATIENT	Master patient index	One record per patient across six hospitals.	Chief data officer	Q4 26	Q1 27	NOW
	Single sign-in	One patient and staff identity everywhere.	Chief digital officer	Q4 26	Q2 27	NOW
OPEN THE DOOR	Online scheduling	Primary care self-booking, then specialty.	Chief patient access officer	Q1 27	Q3 27	NEXT
	Digital intake	Forms, insurance and consent before arrival.	Director of registration	Q2 27	Q4 27	NEXT
PAY AND TRACK	Price estimates	A reliable price shown before the patient books.	VP revenue cycle	Q3 27	Q1 28	LATER
	Referral tracking	One status view from order to appointment.	Chief nursing informatics officer	Q4 27	Q2 28	LATER

Source: Northfield Health System digital front door program.

DECISION	BY	OWNER	WHAT IT UNLOCKS
Fund the data foundation	NOW	Executive team	Wave 1 starts
Approve patient ID policy	NOW	Privacy officer	Single sign-in at every site
Choose buy, build or partner	NEXT	Chief digital officer	Vendor selection and contracts
Set price estimate rules	NEXT	VP revenue cycle	Estimates shown before booking
Agree clinical workflow changes	LATER	Clinical council	Referral tracking and care plans
Decide on enterprise scale-out	LATER	Executive team	Funding for 2028

DIGITAL FRONT DOOR ROADMAP

Six decisions set the pace of the roadmap

A roadmap is a series of choices, each made by a named owner before work can start.

HOW DECISIONS ARE MADE

- **Forum.** Access council, monthly.
- **Rule.** No wave starts without its decision logged.
- **Escalation.** Late decisions move to the executive team within two weeks.

Source: Northfield Health System digital front door program.

NORTH STAR

Eight in ten patients complete access online by 2028.

Baselines are 2026 averages across six hospitals.

DIGITAL FRONT DOOR ROADMAP

Each horizon is judged by its measures

	KPI	BASELINE	TARGET	MOVED BY
NOW	Duplicate patient records	11%	6%	Master patient index
	Sites on one sign-in	0 of 6	6 of 6	Single sign-in
NEXT	Visits booked online	8%	60%	Self-scheduling; digital intake
	Calls to the access center	100%	75%	Reminders; price estimates
LATER	No-show rate	11%	6%	Smart reminders; care plans
	Front-end denials	9%	5%	Digital intake; eligibility checks

Source: Sample figures for layout only; replace with current firm data before use.